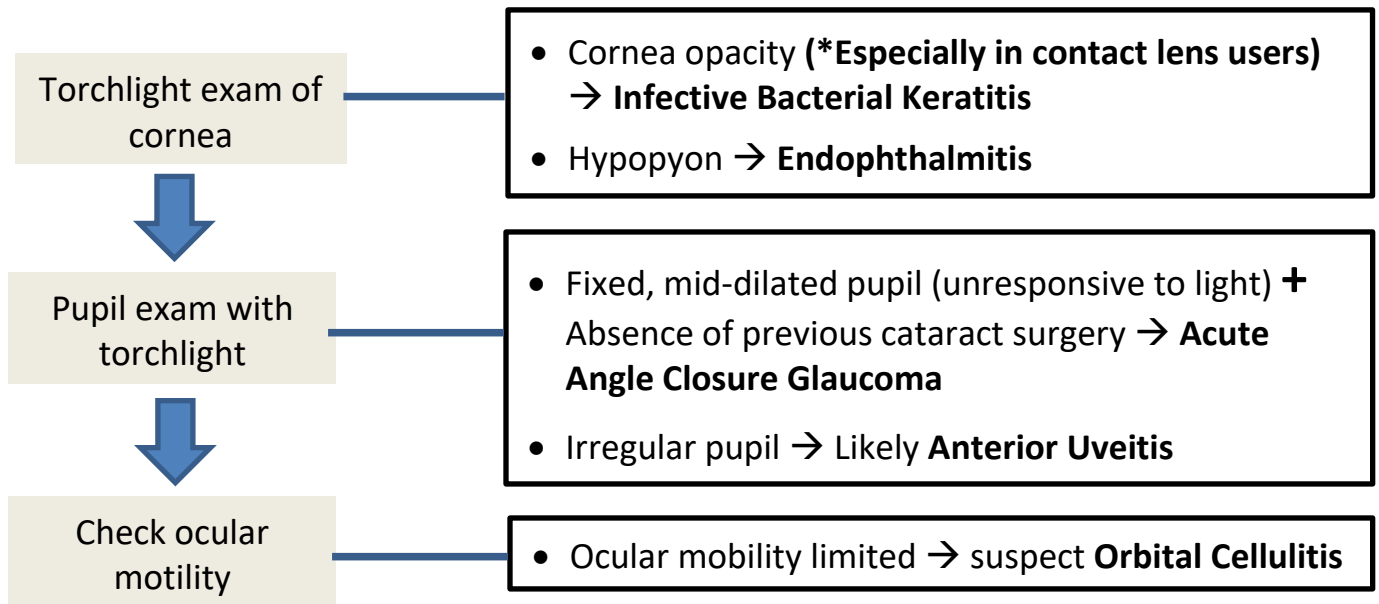


Approach to ACUTE RED EYE

Based on need to rule out etiologies that may cause **early, rapid visual loss** if undiagnosed

Suggested Protocol



1. Refer Stat to Emergency Department or to Ophthalmology Clinic (same day) if:
 - a. Any **positive findings** above OR
 - b. Any cases with **recent ocular intervention** (surgery, intravitreal injections) OR
 - c. Any ***Red Flags**
2. In the absence of All of the above (a-c), one or more of the treatment options below can be instituted where appropriate
 - **Preservative-Free** Lubricating eye drops 1 drop 3HOURLY-PRN (Options below)
 - Refresh ^(\$9.39 per box of 30 single-use vials of 0.4ML)
 - Tears Naturale Free (*Available in NUP retail pharmacy but not in formulary*)
 - Topical antibiotic eye drops 1 drop QDS (Options below)
 - Chloramphenicol for 5 days ^(\$1.10 per bot for SC, \$2.20 per bot for Non-SC) – first line for adult age group
 - Tobramycin for 7 days ^(*Non-Std \$4.00 per bot for SC/Non-SC*) – first line for paediatric age group
 - Ciprofloxacin for 5 days ^(\$5.27 per bot for SC, \$10.54 per bot for Non-SC)
3. Instruct the patient to return in a few days if not getting better or if getting worse
4. **Document:** Clear cornea, no opacity seen. Pupils round, reactive to light. No hypopyon. EOM (extraocular movement) full.

*Red Flags

SUDDEN VISUAL LOSS

NEUROLOGICAL SYMPTOMS (Diplopia, acute ptosis, visual field deficits)

TRAUMA (eye injury, foreign body entry)

POSTERIOR SEGMENT SYMPTOMS (Acute floaters/photopsia)